Better Doctors Better Tomorrow

MEDICOS MINDS

Learn Understand Apply Grow

GENERAL MEDICINE

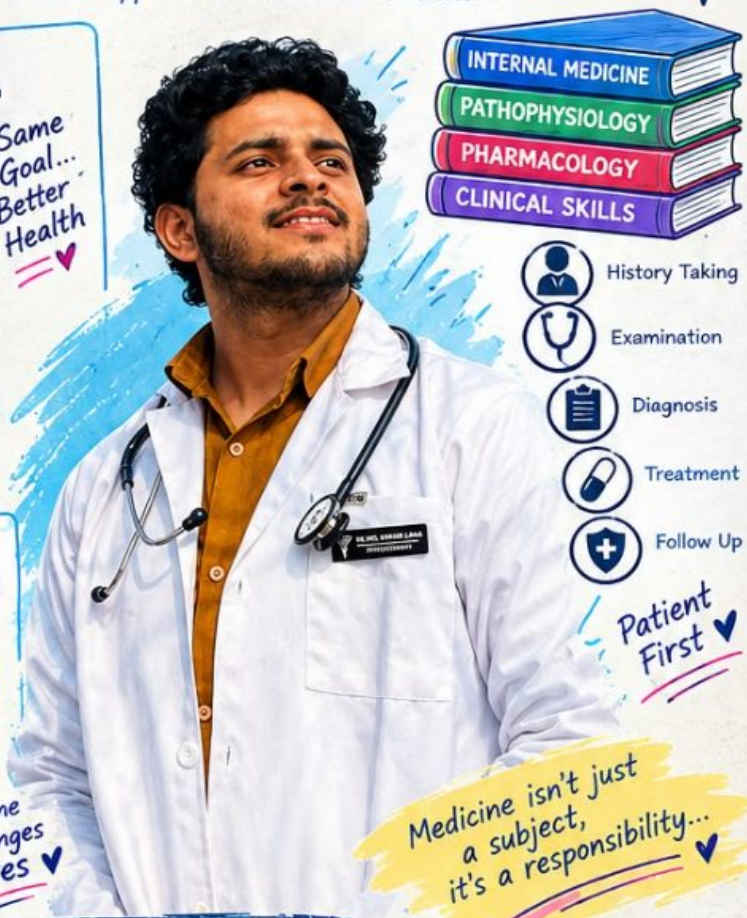
Essentials for Every Physician

Clinical Knowledge • Practical Approach • Better Patient Care

KEY TOPICS

- History Taking & Clinical Examination
- Approach to Common Symptoms
- Cardiovascular System
- Respiratory System
- Gastrointestinal System
- Renal & Electrolyte Disorders
- Endocrinology & Metabolic Disorders
- Hematology
- Infectious Diseases
- Neurology & Mental Health
- Rheumatology
- Oncology
- Geriatrics & Preventive Medicine

Same Goal... Better Health



History Taking

Examination

Diagnosis

Treatment

Follow Up

Patient First

COMMON CONDITIONS

- Diabetes Mellitus
- Hypertension
- COPD / Asthma
- Infections (TB, Pneumonia)
- Heart Failure / MI
- Liver Disease
- Renal Disease
- Thyroid Disorders
- Anemia

Good Medicine Changes Lives

Medicine isn't just a subject, it's a responsibility...

Evidence Based • Patient Centered • Lifelong Learning

MEDICOS_MINDS

NAME : _____

SUBJECT : GENERAL MEDICINE

YEAR : _____

Future Physician Specialist

@restore_healthphysio

- Study
- Practice
- Be a Better Doctor
- Make a Difference

MEDICOS MINDS

HIGH-YIELD BPT STUDY GUIDE & EXAM COMPANION

General Medicine

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OFFICIAL DIGITAL STUDY GUIDE VERIFICATION

Format: High-Yield Digital PDF | Standard Price: Rs. 99

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STUDY GUIDE DETAILS & CONTENTS:

Comprehensive high-yield notes designed for Bachelor of Physiotherapy (BPT), MBBS, BDS, and allied healthcare students. Covers fundamental anatomy, clinical assessments, pathological conditions, evidence-based physical therapy rehabilitation protocols, and exam-oriented high-yield review.

KEY HIGHLIGHTS:

- Exam-Focused: Concise points curated from leading medical textbooks and clinical references.
- Visual Diagrams: High-yield diagrams and tables for quick revision.
- Mobile & Tablet Optimized: Readable on phones, tablets, iPads, and laptops.
- Official Medicos Minds Release: Verified by Dr. MD Kamran Alam.

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GENERAL MEDICINE

Better Knowledge... Better Care... ♡

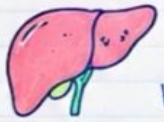
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Stay Curious ♡ Keep Learning...

Liver Health Matters ♡

HEPATITIS

(Viral Hepatitis – B & C)



Healthy Liver = Healthy Life ♡

1. DEFINITION

- **Hepatitis**: Inflammation of the liver.
- **Viral hepatitis**: Caused by hepatotropic viruses (A, B, C, D, E).
- **B & C** are major causes of chronic liver disease, cirrhosis and hepatocellular carcinoma (HCC).

2. TYPES

- **A** – Fecal–oral
- **B** – Blood / sexual / perinatal
- **C** – Blood (mainly), less sexual, perinatal
- **D** – Requires HBV (co-infection)
- **E** – Fecal–oral (water/food)

3. TRANSMISSION (B & C)

- Blood transfusion (unsafe)
- Needle sharing / IVDU
- Unsafe injections / surgery
- Mother to child (vertical)
- Sexual contact (less common for C, common for B)
- Household (rare – no casual contact)

4. CLINICAL FEATURES

- Incubation period: **B** – 6 weeks to 6 months | **C** – 2 to 6 months.
- Often asymptomatic (especially C).
- Symptoms: fatigue, fever, anorexia, nausea, vomiting, abdominal pain, dark urine, jaundice.
- Chronic infection → cirrhosis, liver failure, HCC.

Early detection saves lives ♡

HEPATITIS B (HBV)

1. VIRUS & GENOME

- Double stranded DNA virus (Hepadnavirus).
- Family: Hepadnaviridae.

2. RISK FACTORS

- Unprotected sex, IVDU, multiple partners.
- Blood transfusion, hemodialysis.
- Perinatal transmission (high risk).
- Healthcare workers (needle stick).

3. CLINICAL FEATURES

- Acute → fever, malaise, jaundice, ↑ LFTs.
- Chronic → persistent infection, cirrhosis, HCC.
- Extrahepatic: polyarthritis, PAN, glomerulonephritis.

4. DIAGNOSIS

- **HBsAg** (screening test).
- **HBeAg** (replication & infectivity).
- **HBV DNA** (viral load).
- **Anti-HBc IgM** (acute), IgG (past/chronic).

5. MANAGEMENT

- **Acute**: supportive (rest, hydration).
- **Chronic**: Antiviral therapy (Tenofovir, Entecavir, Peg-IFN).
- **Vaccine**: recombinant HBV vaccine (3 doses).
- **Post-exposure prophylaxis**: HBIG + vaccine (within 24–72 hrs).

Vaccination prevents HBV ♡

HEPATITIS C (HCV)

1. VIRUS & GENOME

- Single stranded RNA virus (Flavivirus).
- Family: Flaviviridae.

2. RISK FACTORS

- Blood transfusion (before screening).
- IVDU (most common).
- Unsafe injections / procedures.
- Hemodialysis.
- Mother to child (less common).

3. CLINICAL FEATURES

- Often asymptomatic.
- **Acute** → mild, self-limiting.
- **Chronic** → 70–80% → chronic hepatitis, cirrhosis, HCC (after 20–30 yrs).
- **Extrahepatic**: cryoglobulinemia, vasculitis, lichen planus.

4. DIAGNOSIS

- Anti-HCV antibody (screening).
- **HCV RNA** (confirmatory & viral load).
- **Genotype** (for treatment).

5. MANAGEMENT

- Direct acting antivirals (DAAs) – high cure rate (> 95%).
- Treat complications (cirrhosis, HCC screening).
- No vaccine available.

Early treatment prevents cirrhosis & HCC

5. DIFFERENCES (Quick View)

Feature	Hepatitis B	Hepatitis C
Genome	DNA	RNA
Chronicity	Yes (5–10%)	Yes (70–80%)
Vaccine	Available	Not available
Treatment	Antivirals + vaccine	DAAs
Risk of HCC	High	High (esp. with cirrhosis)

KEY POINTS

- ☆ Screen all pregnant women for HBsAg & HCV Ab.
- ☆ Vaccinate high-risk individuals (HBV).
- ☆ Early treatment reduces complications.
- ☆ Safe practices = Healthy liver.

Test = Treat = Prevent ♡



"A healthy liver keeps life in balance." ♡

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THANK YOU FOR YOUR PURCHASE!

We wish you the very best in your medical and physiotherapy examinations.

For any questions, supplementary questions, or study support, reach out to us:

SUPPORT & COMMUNITY:

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